Disinhibited Social Engagement Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

Brian Campbell, Ph.D.

Prepared with AI research and drafting assistance; content directed, reviewed, and approved by the author. © 2026 Brian Campbell, Ph.D. All rights reserved.

1. What it is

Disinhibited social engagement disorder (DSED) is a pattern in which a child **actively approaches and interacts with unfamiliar adults** with little or no hesitation, is overly familiar, and may wander off with a stranger, after a history of **severely insufficient care** such as neglect, repeated changes of caregivers, or institutional rearing (American Psychiatric Association [APA], 2022). It belongs to DSM-5-TR's "Trauma- and Stressor-Related Disorders" chapter.

Separate from RAD. DSED and reactive attachment disorder share a cause but look very different: the RAD child is withdrawn, while the DSED child lacks normal caution with strangers and crosses social boundaries. Research supports them as two separate disorders (Zeanah & Gleason, 2015).

More persistent. In a randomized trial of institutionalized Romanian children, disinhibited signs were highest in children who stayed in institutions, lower in foster care, and lowest in children never institutionalized. Unlike withdrawn signs, they improved only partly after placement; placement **before 24 months** was linked to fewer signs (Smyke et al., 2012).

Counselor framing: the child's friendliness can look charming, but it is a safety concern and a sign that the child has not learned that particular adults are special. A loving new home helps, but it does not by itself make these behaviors disappear (Smyke et al., 2012).

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis.

- A pattern of actively approaching and interacting with unfamiliar adults, with **at least 2** of: **reduced or absent reticence** with unfamiliar adults; **overly familiar** verbal or physical behavior (not in keeping with cultural norms or age); **diminished or absent checking back** with the caregiver after venturing away, even in unfamiliar settings; **willingness to go off with an unfamiliar adult** with minimal or no hesitation.
- The behaviors are **not limited to impulsivity** (as in ADHD) but include socially disinhibited behavior.
- A history of **extremes of insufficient care**: persistent lack of comfort and affection, repeated changes of primary caregivers, or rearing in unusual settings such as institutions.
- The insufficient care is presumed to cause the behavior. The child has a developmental age of **at least 9 months**. Specify **persistent** (more than 12 months) and **severe** (all symptoms at relatively high levels).

Rule out: ADHD (impulsivity without the specific social disinhibition and care history), Williams syndrome and other genetic conditions marked by over-friendliness, autism, and intellectual disability (APA, 2022).

Safety and referral (general clinical consensus): DSED leaves children vulnerable to exploitation and abduction. Plan supervision in public and at church, school, and camp. Directly observe the child with caregivers and with unfamiliar adults (Zeanah et al., 2016); refer for pediatric, developmental, and psychiatric evaluation; and report suspected ongoing abuse or neglect.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in disinhibited social engagement disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it. No single test diagnoses DSED; observe the child with caregivers and with unfamiliar adults.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
DAI (Disturbances of Attachment Interview)	Caregiver interview rating signs of RAD and DSED; used in the major research studies.	Clinician interview with the caregiver	No (free)	★ Clinical interviewing skill; training recommended	Original article
RPQ (Relationship Problems Questionnaire)	Brief parent or teacher screen for inhibited and disinhibited attachment behaviors.	Parent- or teacher-report, 10 items	No (free)	None beyond general clinical training	Original article
SDQ (Strengths and Difficulties Questionnaire)	Screening for co-occurring emotional, conduct, attention, and peer problems.	Parent, teacher, and self-report, 25 items	No (free paper version)	None beyond general clinical training	SDQ website
NICHQ Vanderbilt Assessment Scales	Screening for ADHD, which must be distinguished from DSED, ages 6 to 12.	Parent and teacher forms, initial and follow-up	No (free)	None beyond general clinical training	NICHQ (PDF)
BASC-3 (Behavior Assessment System for Children)	Broad behavioral and emotional assessment, ages 2 to 21.	Parent, teacher, and self-report forms	Yes	★ Level B	Pearson
TSCYC (Trauma Symptom Checklist for Young Children)	Caregiver report of trauma symptoms in children ages 3 to 12.	Caregiver-report, 90 items (screening form 20)	Yes	★ Level B	PAR

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: placement in a stable, sensitive family helps, but DSED improves less readily than RAD. Treatment combines caregiver-focused work with explicit teaching and practice of social boundaries. Coercive "attachment therapies" should be avoided.

Evidence overview (honest effect sizes):

- **Family placement helps, early is better:** foster care lowered disinhibited signs compared with continued institutional care, and placement before 24 months predicted fewer signs, though they often persisted (Smyke et al., 2012).
- **Treatment aims:** the practice parameter calls for improving disturbed social relatedness toward non-caregivers as well as strengthening the attachment relationship (Zeanah et al., 2016). Specific treatment trials for DSED are lacking (Zeanah & Gleason, 2015).
- **Building caregiver sensitivity:** across 70 studies, brief, behaviorally focused interventions improved parental sensitivity (d = 0.33) and infant attachment security (d = 0.20) (Bakermans-Kranenburg et al., 2003). These studies did not diagnose DSED itself.
- **Avoid coercive therapies:** the benefits and risks of many "attachment therapies" are unproven, and some techniques used with foster and adopted children are potentially harmful (Chaffin et al., 2006).
- **Medication (referral notes):** no medication treats DSED. Medication may help co-occurring ADHD.

Cognitive-behavioral work with DSED: how it is used (Zeanah et al., 2016):

- **Teach and practice boundaries:** "safe adults" versus strangers, asking a parent first, staying close in public, and appropriate greetings (a wave or handshake rather than a hug), with role-play and praise.
- **Identify and challenge caregiver beliefs:** "He's just very friendly," "She doesn't need me," "If he really loved us, he wouldn't go to strangers."
- **Strengthen the special relationship** with caregivers through predictable routines, checking-in games, and warm responses when the child returns to the parent.

5. Religion, spirituality, and Christian integration: what the evidence shows

Disorder-specific research. We found **no study of religion, faith communities, or faith-integrated treatment in DSED** (literature search, November 1, 2026).

© 2026 Brian Campbell, Ph.D. All rights reserved.

Related evidence. Across 49 studies, positive religious coping went with better adjustment to stress and negative religious coping with worse (Ano & Vasconcelles, 2005). Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

1. A stable, sensitive family, early placement, caregiver-focused work, and boundary teaching are the best available approach. Faith integration supports them; it does not replace them.
2. There is no research on faith and DSED. Church settings, with many friendly adults, need clear safeguarding plans for these children.
3. Unproven and coercive "attachment therapies" have sometimes been offered to foster and adoptive families (Chaffin et al., 2006). Guide families toward evidence-based help.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method mainly with caregivers. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for caregiver beliefs that minimize risk or wound the parent: "Everyone at church is safe," "His friendliness is a gift," "She would rather be with anyone than me." With older children, listen for "Any grown-up will take care of me" (Smyke et al., 2012). To identify deeper false beliefs, see: Core Beliefs, Parental Messages, or use the Semantic Search Tool.
- **Challenge (clinical).** Separate warmth toward others from safety. Track how often the child checks back with the parent and celebrate progress. **Possible cognitive errors** to watch for, mainly in caregivers, with their definitions from the Belief Counseling Cognitive Errors Checklist and the problems they tend to feed:
  - **Jumping to conclusions** — deciding without the facts. *Feeds:* lax supervision. *Sounds like:* "Everyone at church is safe."
  - **Mind reading** — assuming I know what others think. *Feeds:* parental hurt. *Sounds like:* "She doesn't need me."
  - **Personalization** — taking blame for things outside my control. *Feeds:* parental discouragement. *Sounds like:* "He goes to strangers because I'm a bad mother."
  - **All-or-nothing thinking** — no middle ground. *Feeds:* parental hurt. *Sounds like:* "If he loved us, he'd never go to strangers."
  - **Fortune telling** — predicting a bad outcome. *Feeds:* giving up on boundary teaching. *Sounds like:* "He'll never learn to stay close."
- **Questions to dispute the belief.** Have the parent (or an older child) choose questions from the Questioning False Beliefs form that "speak to" the belief. To see how, watch Dr. Campbell's video Integrated Christian Counseling: Questioning Your Beliefs. Selected questions that fit the errors above:
  - "What's the actual evidence for my belief?" *For:* jumping to conclusions.
  - "Are there other ways of looking at this situation?" *For:* mind reading.
  - "Am I really the cause of the problem, or did someone just try to convince me that I am?" and "Am I being too hard on myself?" *For:* personalization.
  - "Am I keeping things in perspective?" *For:* all-or-nothing thinking.
  - "Can I really predict the future?" *For:* fortune telling.
- **Replace (pastoral).** Offer Scripture of belonging and of wise protection:
  - "Children are a heritage from the LORD, offspring a reward from Him." (Psalm 127:3). This child is entrusted to these parents in particular.
  - "I am the good shepherd. I know My sheep and My sheep know Me." (John 10:14). Learning to know and return to one's own caregivers.
  - "The prudent see danger and take cover, but the simple keep going and suffer the consequences." (Proverbs 22:3). Teaching caution is loving, not unkind.
  - "God settles the lonely in families…" (Psalm 68:6)
- **Faith community (clinical).** Brief nursery, Sunday school, and youth workers on the child's safety plan; use check-in and pickup procedures; and ask adults to redirect the child to the parent rather than accepting hugs or wandering off.
- **Ministers:** support church safeguarding policies, avoid praising the child's indiscriminate friendliness, and refer families to clinicians experienced with foster and adopted children.

© 2026 Brian Campbell, Ph.D. All rights reserved.

7. Quick classroom takeaways

- DSED means a child who approaches and goes off with unfamiliar adults without normal caution, after severe neglect or disrupted care. It is separate from RAD and from ADHD impulsivity.
- Family placement helps, especially before age 2, but signs often persist; add boundary teaching and close supervision. Avoid coercive therapies.
- Replace "any adult will care for me" with belonging to one's own family, and make church a safe place with clear safeguards.
- Diagnose through caregiver interview (such as the DAI) and observation of how the child approaches strangers; rule out ADHD with the Vanderbilt scales.

Sources note

All 8 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders* (5th ed., text rev.). https://doi.org/10.1176/appi.books.9780890425787

Ano, G. G., & Vasconcelles, E. B. (2005). Religious coping and psychological adjustment to stress: A meta-analysis. *Journal of Clinical Psychology*, *61*(4), 461–480. https://doi.org/10.1002/jclp.20049

Bakermans-Kranenburg, M. J., van IJzendoorn, M. H., & Juffer, F. (2003). Less is more: Meta-analyses of sensitivity and attachment interventions in early childhood. *Psychological Bulletin*, *129*(2), 195–215. https://doi.org/10.1037/0033-2909.129.2.195

Captari, L. E., Hook, J. N., Hoyt, W. T., Davis, D. E., McElroy-Heltzel, S. E., & Worthington, E. L., Jr. (2018). Integrating clients' religion and spirituality within psychotherapy: A comprehensive meta-analysis. *Journal of Clinical Psychology*, *74*(11), 1938–1951. https://doi.org/10.1002/jclp.22681

Chaffin, M., Hanson, R., Saunders, B. E., Nichols, T., Barnett, D., Zeanah, C., Berliner, L., Egeland, B., Newman, E., Lyon, T., Letourneau, E., & Miller-Perrin, C. (2006). Report of the APSAC task force on attachment therapy, reactive attachment disorder, and attachment problems. *Child Maltreatment*, *11*(1), 76–89. https://doi.org/10.1177/1077559505283699

Smyke, A. T., Zeanah, C. H., Gleason, M. M., Drury, S. S., Fox, N. A., Nelson, C. A., & Guthrie, D. (2012). A randomized controlled trial comparing foster care and institutional care for children with signs of reactive attachment disorder. *American Journal of Psychiatry*, *169*(5), 508–514. https://doi.org/10.1176/appi.ajp.2011.11050748

Zeanah, C. H., & Gleason, M. M. (2015). Annual research review: Attachment disorders in early childhood—clinical presentation, causes, correlates, and treatment. *Journal of Child Psychology and Psychiatry*, *56*(3), 207–222. https://doi.org/10.1111/jcpp.12347

Zeanah, C. H., Chesher, T., Boris, N. W., & the American Academy of Child and Adolescent Psychiatry Committee on Quality Issues. (2016). Practice parameter for the assessment and treatment of children and adolescents with reactive attachment disorder and disinhibited social engagement disorder. *Journal of the American Academy of Child and Adolescent Psychiatry*, *55*(11), 990–1003. https://doi.org/10.1016/j.jaac.2016.08.004

Prepared for Clinical Companion (Brian Campbell). Teaching overview only. Clinically review before use. Not a substitute for the DSM-5-TR or clinical judgment. All 8 references and DOIs verified via Crossref, OpenAlex, and PubMed/Europe PMC records (Sunday, Nov 1, 2026). Effect sizes as reported in the cited sources.

© 2026 Brian Campbell, Ph.D. All rights reserved.